

CLINICAL INTELLIGENCE TREND REPORT

YELLOW: Closer Observation Suggested

Patient ID: HarrisPeriod: April 1 to April 30, 2026Report Date: May 1, 2026Coverage: 659/720h (92%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

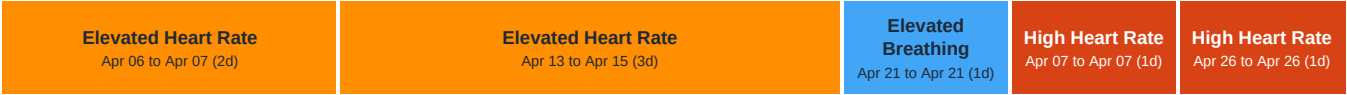
High HR > 100 bpm

Very High Breathing > 40 brpm

Patient clinical status over 30 days from Apr 01 to Apr 30

HR

Breathing



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: YELLOW

Vital	Avg	Min	Max
Heart Rate	77 bpm	48 bpm	109 bpm
Breathing	17 brpm	12 brpm	40 brpm

Last 24 hours: no episodic events, vitals within normal range.

Episodic Events (last 24h)

No episodic events in the last 24 hours.

12 episodic events Detected, spanning 20h total. Conditions: Elevated Breathing, Elevated Heart Rate, High Heart Rate. Priority grade: Medium

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
12	20h	Elevated Heart Rate	<1	92%

Major Findings: Sustained elevated HR (avg 98, peak 111)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 07	12 AM to 1 AM	1h	1	High Heart Rate	104	80	110	Check temp, hydration, pain; review for arrhythmia or infection
Apr 26	1 AM to 2 AM	1h	1	High Heart Rate	100	87	111	Check temp, hydration, pain; review for arrhythmia or infection
Apr 06	7 PM to 12 AM	11h	5	Elevated Heart Rate	98	72	109	Check temp, hydration, pain; review for arrhythmia or infection
Apr 13	1 AM to 4 AM	6h	4	Elevated Heart Rate	97	54	104	Check temp, hydration, pain; review for arrhythmia or infection
Apr 21	11 AM to 12 PM	1h	1	Elevated Breathing	24	18	35	Check SpO2 and lung sounds; assess for fluid overload or early infection

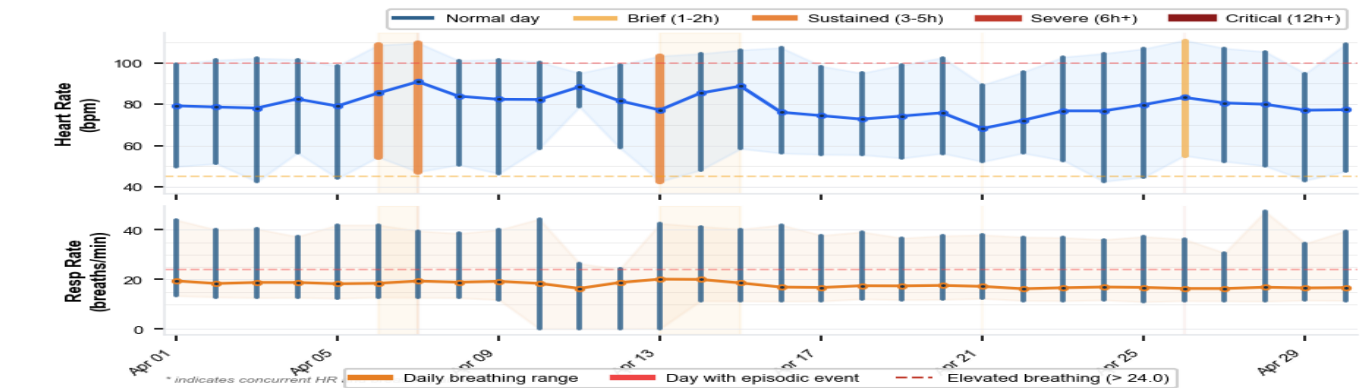
The P5 to P95 heart rate spread was 28 bpm (65 to 93), indicating significant cardiac variability.

Suggested Clinical Review Actions

- Elevated Heart Rate (avg 98 bpm, peak 109 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Breathing (avg 24 brpm, peak 35 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Heart Rate (avg 104 bpm, peak 110 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Overall trajectory shows 5 unstable phases with 28 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

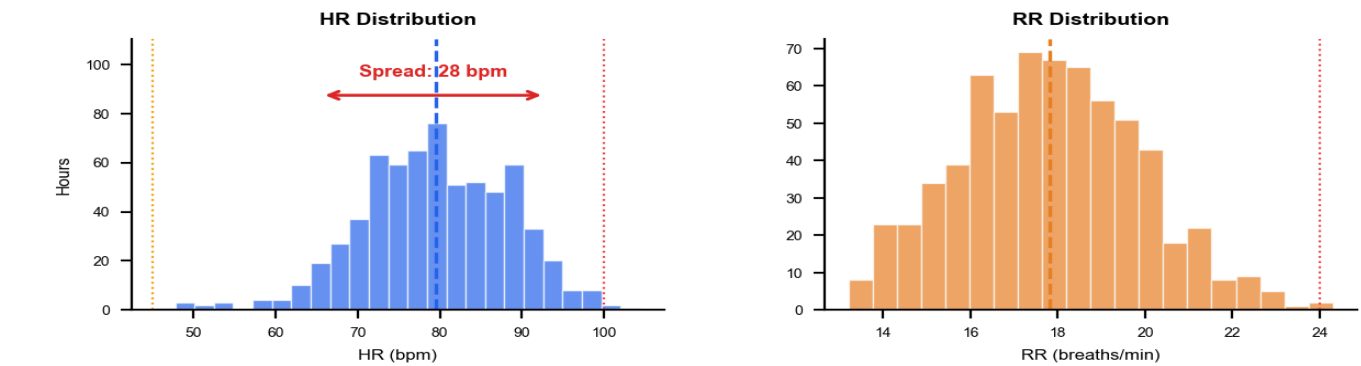
Clinical Pattern Observations:

- 1. High variability: HR spread 27 bpm (65 to 93).
- 2. Clustered pattern: Episodic events on 4 of 30 days (13%).
- 3. Nocturnal pattern: 5 of 6 eligible episodes during evening or night hours.

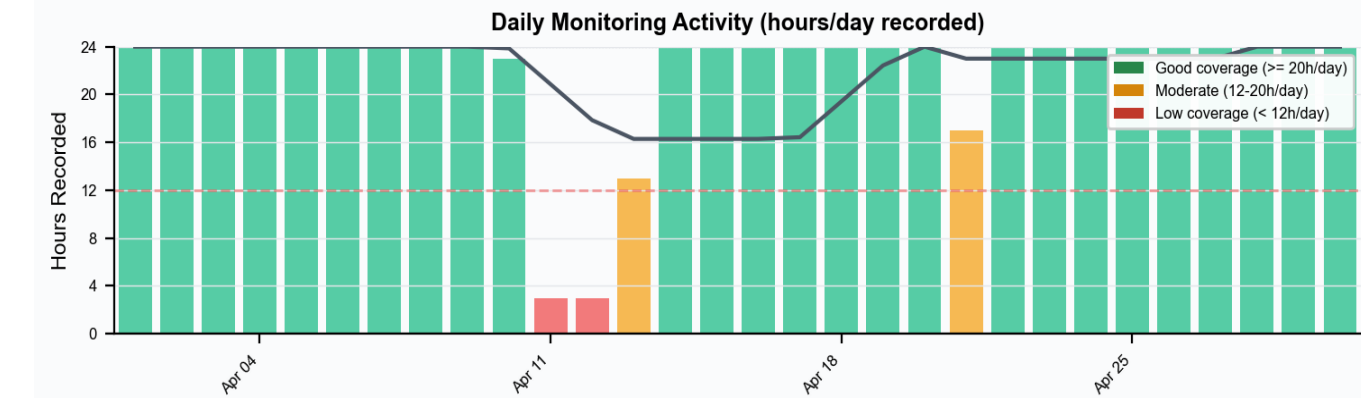


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

HR episode, RR episode, Recorded, no episode, No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.